

Undergraduate Medicine

Gastroenterology Study Pack

CharaNas Medicine — Specialty Notes

Expanded notes with original schematic figures for revision. These are educational drawings inspired by standard undergraduate teaching themes, not copied textbook plates. Always follow your faculty curriculum and latest guidelines.

Section	Focus
1	Approach to abdominal pain
2	Upper GI & H. pylori
3	Hepatobiliary disease
4	Pancreas & bowel
5	Checklist & books
6	Quick pearls from these notes
7	Recommended book sources

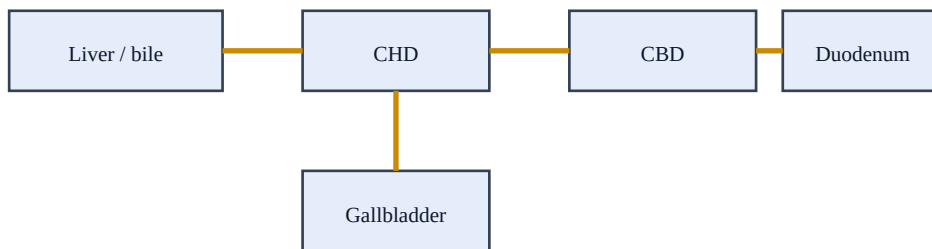
1. Approach to abdominal pain

Characterize pain: onset, site, radiation, character, severity, relation to meals/defecation, associated vomiting/bleeding/fever/jaundice.

Surgical abdomen red flags: peritonitis (guarding/rigidity), intractable vomiting, hemodynamic instability, suspected obstruction/perforation.

Common UG diagnoses: appendicitis, cholecystitis, pancreatitis, PUD, diverticulitis, gastroenteritis, biliary colic, mesenteric ischemia (older/AF).

Figure: Biliary tree pathway (schematic)



Obstruction + infection → ascending cholangitis (Charcot triad)

Schematic figure for learning — verify details in your recommended textbooks.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about gastroenterology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

2. Upper GI & H. pylori

Dyspepsia and peptic ulcer disease often link to H. pylori and NSAIDs. Alarm features (weight loss, anemia, dysphagia, bleeding, age thresholds) warrant endoscopy pathways.

Upper GI bleed: resuscitation first, PPI per protocol, risk scores, endoscopy timing. Think varices in liver disease — airway and volume care.

GERD is common; persistent symptoms or alarm features need further evaluation.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about gastroenterology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

3. Hepatobiliary disease

Charcot triad (RUQ pain, fever, jaundice) = ascending cholangitis until proven otherwise. Reynolds pentad adds hypotension and confusion.

Cholangitis needs antibiotics and biliary drainage (often ERCP). Obstructive LFTs show predominant ALP/GGT and bilirubin rise.

Cirrhosis complications: variceal bleed, ascites, SBP, encephalopathy, hepatorenal syndrome, HCC surveillance.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about gastroenterology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

4. Pancreas & bowel

Acute pancreatitis: epigastric pain to back + elevated lipase. Gallstones and alcohol are leading causes. Early care is supportive fluids/analgesia; severity scoring guides monitoring.

Appendicitis: periumbilical pain migrating to RLQ, anorexia, McBurney's tenderness. Imaging when atypical.

IBD vs IBS: alarm features, nocturnal diarrhea, bleeding, weight loss, anemia point away from simple IBS.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about gastroenterology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

5. Checklist & books

Checklist: vitals, abdominal exam (including hernial orifices), pregnancy test, FBC/LFTs/lipase, AXR/US/CT as indicated, sepsis care if needed.

Books: Sleisenger & Fordtran; Harrison's GI; Bailey & Love abdominal chapters.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about gastroenterology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

6. Quick pearls

High-yield reminders packaged with this specialty in CharaNas:

1. Charcot triad = cholangitis; Reynolds pentad adds hypotension + confusion.
2. *H. pylori* → PUD; test-and-treat in appropriate settings.
3. Pancreatitis: epigastric pain to back + ↑ lipase; fluids and support.
4. Appendicitis: periumbilical→RLQ pain, McBurney's tenderness.
5. Cirrhosis complications: varices, ascites, encephalopathy, HCC surveillance.
 - Link symptoms → anatomy/physiology → investigation → first management step.
 - Write one OSCE-style explanation for a common presentation in this specialty.
 - After reading, do the Short MCQ and Case-based Question for active recall.

7. Recommended book sources

Standard undergraduate references commonly used internationally. Use the edition your college recommends. Figures in commercial textbooks are copyrighted; use library/licensed access for full plates and photographs.

#	Reference
1	Sleisenger and Fordtran's Gastrointestinal and Liver Disease
2	Harrison's — Gastroenterology
3	Bailey & Love — Abdominal surgery chapters

How to study with this PDF: read one section → sketch the figure from memory → answer the MCQs/cases → review mistakes → revisit the matching section.

Disclaimer: educational aid only; not a substitute for clinical guidelines, faculty teaching, or licensed textbooks.